| PATIENT NAME : TARUN KUMAR | | REF. DOCTOR : SELF |
|---|---|---|
| CODE/NAME & ADDRESS : C000133041<br>CHECKMED PRIVATE LIMITED<br>SECOND FLOOR, SCO 334-334, SECTOR 35,CHANDIGARH<br>CHANDIGARH 160022<br>9914256267 | ACCESSION NO : **0282YH000247**<br>PATIENT ID : TARUM230691282<br>CLIENT PATIENT ID:<br>ABHA NO : | AGE/SEX : 34 Years Male<br>DRAWN :<br>RECEIVED : 06/08/2025 13:23:28<br>REPORTED : 07/08/2025 09:54:06 |

| Test Report Status **Final** | Results | Biological Reference Interval | Units |
|---|---|---|---|

**ANNUAL HEALTH CHECK-UP PACKAGE**

**ECG**

| | |
|---|---|
| ECG | WITHIN NORMAL LIMITS |

**TMT**

| | |
|---|---|
| STRESS TEST | STRESS TEST IS NEGATIVE FOR RMI |

**MEDICAL HISTORY**

| | |
|---|---|
| RELEVANT PRESENT HISTORY | NOT SIGNIFICANT |
| RELEVANT PAST HISTORY | NOT SIGNIFICANT |
| RELEVANT PERSONAL HISTORY | VEG. SMOKER, ALCOHOL OCC. |
| RELEVANT FAMILY HISTORY | MOTHER - HTN |
| OCCUPATIONAL HISTORY | SERVICE |
| HISTORY OF MEDICATIONS | NOT SIGNIFICANT |

**ANTHROPOMETRIC DATA & BMI**

| ANTHROPOMETRIC DATA & BMI | Results | 0282XA000026<br>02/01/24 09:24 | | | Biological Reference Interval | Units |
|---|---|---|---|---|---|---|
| HEIGHT IN METERS | 1.83 | 1.83 | | | | mts |
| WEIGHT IN KGS. | 101 | 97.7 | | | | Kgs |
| BMI | 30 | | | | BMI & Weight Status as follows:<br>Below 18.5: Underweight<br>18.5 - 24.9: Normal<br>25.0 - 29.9: Overweight<br>30.0 and Above: Obese | kg/sqmts |



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Haryana, India
Tel : 9111591115

ULR No.775000013678494-0282

| | | |
|---|---|---|
| **PATIENT NAME : TARUN KUMAR** | **REF. DOCTOR :** SELF | |
| **CODE/NAME & ADDRESS :** C000133041<br>CHECKMED PRIVATE LIMITED<br>SECOND FLOOR, SCO 334-334, SECTOR 35,CHANDIGARH<br>CHANDIGARH 160022<br>9914256267 | ACCESSION NO : **0282YH000247**<br>PATIENT ID : TARUM230691282<br>CLIENT PATIENT ID:<br>ABHA NO : | AGE/SEX : 34 Years Male<br>DRAWN :<br>RECEIVED : 06/08/2025 13:23:28<br>REPORTED : 07/08/2025 09:54:06 |

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|---|---|---|---|

**GENERAL EXAMINATION**

| | |
|---|---|
| MENTAL / EMOTIONAL STATE | NORMAL |
| PHYSICAL ATTITUDE | NORMAL |
| GENERAL APPEARANCE / NUTRITIONAL STATUS | OVERWEIGHT |
| BUILT / SKELETAL FRAMEWORK | AVERAGE |
| FACIAL APPEARANCE | NORMAL |
| SKIN | NORMAL |
| UPPER LIMB | NORMAL |
| LOWER LIMB | NORMAL |
| NECK | NORMAL |
| NECK LYMPHATICS / SALIVARY GLANDS | NOT ENLARGED OR TENDER |
| THYROID GLAND | NOT ENLARGED |
| CAROTID PULSATION | NORMAL |
| TEMPERATURE | NORMAL |
| PULSE | 71 / MIN REGULAR, ALL PERIPHERAL PULSES WELL FELT |
| RESPIRATORY RATE | NORMAL |

| **CARDIOVASCULAR SYSTEM** | **Results** | **0282XA000026**<br>02/01/24 09:24 | | | **Biological Reference Interval** | **Units** |
|---|---|---|---|---|---|---|
| BP | 121/81 MMHG (SITTING) | 118/80 MM HG (SUPINE) | | | | mm/Hg |
| PERICARDIUM | NORMAL | NORMAL | | | | |
| APEX BEAT | NORMAL | NORMAL | | | | |
| HEART SOUNDS | S1, S2 HEARD NORMALLY | S1, S2 HEARD NORMALLY | | | | |
| MURMURS | ABSENT | ABSENT | | | | |



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|---|---|---|---|

**RESPIRATORY SYSTEM**

| | |
|---|---|
| SIZE AND SHAPE OF CHEST | NORMAL |
| MOVEMENTS OF CHEST | SYMMETRICAL |
| BREATH SOUNDS INTENSITY | NORMAL |
| BREATH SOUNDS QUALITY | VESICULAR (NORMAL) |
| ADDED SOUNDS | ABSENT |

**PER ABDOMEN**

| | |
|---|---|
| APPEARANCE | NORMAL |
| VENOUS PROMINENCE | ABSENT |
| LIVER | NOT PALPABLE |
| SPLEEN | NOT PALPABLE |

**CENTRAL NERVOUS SYSTEM**

| | |
|---|---|
| HIGHER FUNCTIONS | NORMAL |
| CRANIAL NERVES | NORMAL |
| CEREBELLAR FUNCTIONS | NORMAL |
| SENSORY SYSTEM | NORMAL |
| MOTOR SYSTEM | NORMAL |
| REFLEXES | NORMAL |

**MUSCULOSKELETAL SYSTEM**

| | |
|---|---|
| SPINE | NORMAL |
| JOINTS | NORMAL |



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**BASIC EYE EXAMINATION**

| | |
|---|---|
| DISTANT VISION RIGHT EYE WITH GLASSES | 6/6 |
| DISTANT VISION LEFT EYE WITH GLASSES | 6/6 |
| NEAR VISION RIGHT EYE WITH GLASSES | N/6 |
| NEAR VISION LEFT EYE WITH GLASSES | N/6 |
| COLOUR VISION | 17/17 |

**SUMMARY**

| | |
|---|---|
| REMARKS / RECOMMENDATIONS | DYSLIPIDEMIA<br><br>ADVISED<br>REGULAR EXERCISE<br>LIFESTYLE MODIFICATION<br>DIETARY CHANGES<br>FOLLOW UP WITH PHYSICIAN |

***This is a tabular depiction of test results illustrating values of the previous three reports of the patient which needs to be correlated with the original reports**



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|---|---|---|

**ANNUAL HEALTH CHECK-UP PACKAGE**

**SCREENING ECHOCARDIOGRAM**

**CLINICAL PROFILE**

ECHO REPORT

- **Normal sized cardiac chambers and normal valves**
- **No RWMA**
- **Trivial TR**
- **Normal LV/RV systolic function LVEF ~ 60 %**
- **Normal MIP**
- **No Clot/Vegetation/Pericardial Effusion**
- **IVS/IAS intact, no flow seen across.**

**ULTRASOUND ABDOMEN**

**ULTRASOUND ABDOMEN**

**U.S.G Scan S/o Mild hepatomegaly with grade II fatty liver.**
**Splenomegaly with A small tiny hypoechoic lesion seen in its parenchyma - ?cyst.**

*Please correlate clinically.*

**Interpretation(s)**
MEDICAL
HISTORY-**********
THIS REPORT CARRIES THE SIGNATURE OF OUR LABORATORY DIRECTOR. THIS IS AN INVIOLABLE FEATURE OF OUR LAB MANAGEMENT SOFTWARE. HOWEVER, ALL EXAMINATIONS AND INVESTIGATIONS HAVE BEEN CONDUCTED BY OUR PANEL OF DOCTORS.

**********

****End Of Report****
**Please visit www.agilusdiagnostics.com for related Test Information for this accession**



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| | | |
|---|---|---|
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**Test Report Status** **Final**

## HAEMATOLOGY - CBC

**ANNUAL HEALTH CHECK-UP PACKAGE**

| **BLOOD COUNTS, EDTA WHOLE BLOOD** | **Results** | **0282XA000026** 02/01/24 09:24 | | | **Biological Reference Interval** | **Units** |
|---|---|---|---|---|---|---|
| HEMOGLOBIN (HB)<br>METHOD : SPECTROPHOTOMETRY | 14.3 | | | | 13.0 - 17.0 | g/dL |
| RED BLOOD CELL (RBC) COUNT<br>METHOD : ELECTRICAL IMPEDANCE | 6.29 H | | | | 4.5 - 5.5 | mil/µL |
| WHITE BLOOD CELL (WBC) COUNT<br>METHOD : IMPEDANCE | 6.75 | | | | 4 - 10 | thou/µL |
| PLATELET COUNT<br>METHOD : IMPEDANCE | 217 | | | | 150 - 410 | thou/µL |

| **RBC AND PLATELET INDICES** | **Results** | **0282XA000026** 02/01/24 09:24 | | | **Biological Reference Interval** | **Units** |
|---|---|---|---|---|---|---|
| HEMATOCRIT (PCV)<br>METHOD : CALCULATED | 44.4 | | | | 40.0 - 50.0 | % |
| MEAN CORPUSCULAR VOLUME (MCV)<br>METHOD : DERIVED FROM IMPEDANCE MEASURE | 70.7 L | | | | 83.0 - 101.0 | fL |
| MEAN CORPUSCULAR HEMOGLOBIN (MCH)<br>METHOD : CALCULATED PARAMETER | 22.7 L | | | | 27.0 - 32.0 | pg |
| MEAN CORPUSCULAR HEMOGLOBIN CONCENTRATION(MCHC)<br>METHOD : CALCULATED PARAMETER | 32.1 | | | | 31.5 - 34.5 | g/dL |
| RED CELL DISTRIBUTION WIDTH (RDW) | 17.1 H | | | | 11.6 - 14.0 | % |

K. I. Prajapati



**Dr. Prajapati Kamlesh Ishwarbhai**
**Consultant - Pathologist**

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Haryana, India
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**ULR No.775000013678494-0009**

| PATIENT NAME : TARUN KUMAR | | REF. DOCTOR : SELF |
|---|---|---|
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|---|---|---|---|
| METHOD : DERIVED FROM IMPEDANCE MEASURE | | | |
| MEAN PLATELET VOLUME (MPV) | 11.1 H | 6.8 - 10.9 | fL |
| METHOD : DERIVED FROM IMPEDANCE MEASURE | | | |

| WBC DIFFERENTIAL COUNT | Results | 0282XA000026<br>02/01/24 09:24 | Biological Reference Interval | Units |
|---|---|---|---|---|
| NEUTROPHILS | 52 | 40 - 80 | % |
| METHOD : DHSS FLOWCYTOMETRY | | | |
| LYMPHOCYTES | 34 | 20 - 40 | % |
| METHOD : DHSS FLOWCYTOMETRY | | | |
| MONOCYTES | 7 | 2.0 - 10.0 | % |
| METHOD : DHSS FLOWCYTOMETRY | | | |
| EOSINOPHILS | 7 H | 1 - 6 | % |
| METHOD : DHSS FLOWCYTOMETRY | | | |
| BASOPHILS | 0 | 0 - 2 | % |
| METHOD : IMPEDANCE | | | |
| ABSOLUTE NEUTROPHIL COUNT | 3.50 | 2 - 7 | thou/µL |
| METHOD : CALCULATED | | | |
| ABSOLUTE LYMPHOCYTE COUNT | 2.30 | 1.0 - 3.0 | thou/µL |
| METHOD : CALCULATED | | | |
| ABSOLUTE MONOCYTE COUNT | 0.44 | 0.20 - 1.00 | thou/µL |
| METHOD : CALCULATED | | | |
| ABSOLUTE EOSINOPHIL COUNT | 0.47 | 0.02 - 0.50 | thou/µL |
| METHOD : CALCULATED | | | |
| ABSOLUTE BASOPHIL COUNT | 0.00 | 0.0 - 0.10 | thou/µL |
| METHOD : CALCULATED | | | |

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|---|---|---|---|

***This is a tabular depiction of test results illustrating values of the previous three reports of the patient which needs to be correlated with the original reports**

**Interpretation(s)**
RBC AND PLATELET INDICES-Mentzer index (MCV/RBC) is an automated cell-counter based calculated screen tool to differentiate cases of Iron deficiency anaemia(>13) from Beta thalassaemia trait (<13) in patients with microcytic anaemia. This needs to be interpreted in line with clinical correlation and suspicion. Estimation of HbA2 remains the gold standard for diagnosing a case of beta thalassaemia trait.
WBC DIFFERENTIAL COUNT-The optimal threshold of 3.3 for NLR showed a prognostic possibility of clinical symptoms to change from mild to severe in COVID positive patients. When age = 49.5 years old and NLR = 3.3, 46.1% COVID-19 patients with mild disease might become severe. By contrast, when age < 49.5 years old and NLR < 3.3, COVID-19 patients tend to show mild disease. (Reference to - The diagnostic and predictive role of NLR, d-NLR and PLR in COVID-19 patients A.-P. Yang, et al. International Immunopharmacology 84 (2020) 106504
This ratio element is a calculated parameter and out of NABL scope.

K. I. Prajapati



**Dr. Prajapati Kamlesh Ishwarbhai**
**Consultant - Pathologist**

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|---|---|---|---|

## HAEMATOLOGY

**ANNUAL HEALTH CHECK-UP PACKAGE**

| GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD | Results | 0282XA000026<br>02/01/24 09:24 | | | Biological Reference Interval | Units |
|---|---|---|---|---|---|---|
| HBA1C | 5.6 | | | | Non-diabetic: < 5.7<br>Pre-diabetics: 5.7 - 6.4<br>Diabetics: > or = 6.5<br>ADA Target: 7.0<br>Action suggested: > 8.0 | % |
| METHOD : HIGH PERFORMANCE LIQUID CHROMATOGRAPHY | | | | | | |
| ESTIMATED AVERAGE GLUCOSE(EAG) | 114.0 | | | | < 116 | mg/dL |
| METHOD : CALCULATED PARAMETER | | | | | | |

***This is a tabular depiction of test results illustrating values of the previous three reports of the patient which needs to be correlated with the original reports**

**Interpretation(s)**
GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD-**Used For**:

1. Evaluating the long-term control of blood glucose concentrations in diabetic patients.
2. Diagnosing diabetes.
3. Identifying patients at increased risk for diabetes (prediabetes).

The ADA recommends measurement of HbA1c (typically 3-4 times per year for type 1 and poorly controlled type 2 diabetic patients, and 2 times per year for well-controlled type 2 diabetic patients) to determine whether a patients metabolic control has remained continuously within the target range.

1. eAG (Estimated average glucose) converts percentage HbA1c to md/dl, to compare blood glucose levels.
2. eAG gives an evaluation of blood glucose levels for the last couple of months.
3. eAG is calculated as eAG (mg/dl) = 28.7 * HbA1c - 46.7

**HbA1c Estimation can get affected due to :**

1. Shortened Erythrocyte survival : Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g. recovery from acute blood loss,hemolytic anemia) will falsely lower HbA1c test results.Fructosamine is recommended in these patients which indicates diabetes control over 15 days.
2.Vitamin C & E are reported to falsely lower test results.(possibly by inhibiting glycation of hemoglobin.
3. Iron deficiency anemia is reported to increase test results. Hypertriglyceridemia,uremia, hyperbilirubinemia, chronic alcoholism,chronic ingestion of salicylates & opiates addiction are reported to interfere with some assay methods,falsely increasing results.
4. Interference of hemoglobinopathies in HbA1c estimation is seen in

a) Homozygous hemoglobinopathy. Fructosamine is recommended for testing of HbA1c.
b) Heterozygous state detected (D10 is corrected for HbS & HbC trait.)
c) HbF > 25% on alternate paltform (Boronate affinity chromatography) is recommended for testing of HbA1c.Abnormal Hemoglobin electrophoresis (HPLC method) is recommended for detecting a hemoglobinopathy

K. I. Prajapati



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**Consultant - Pathologist**

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**ULR No.775000013678494-0009**

| PATIENT NAME : TARUN KUMAR | | REF. DOCTOR : SELF |
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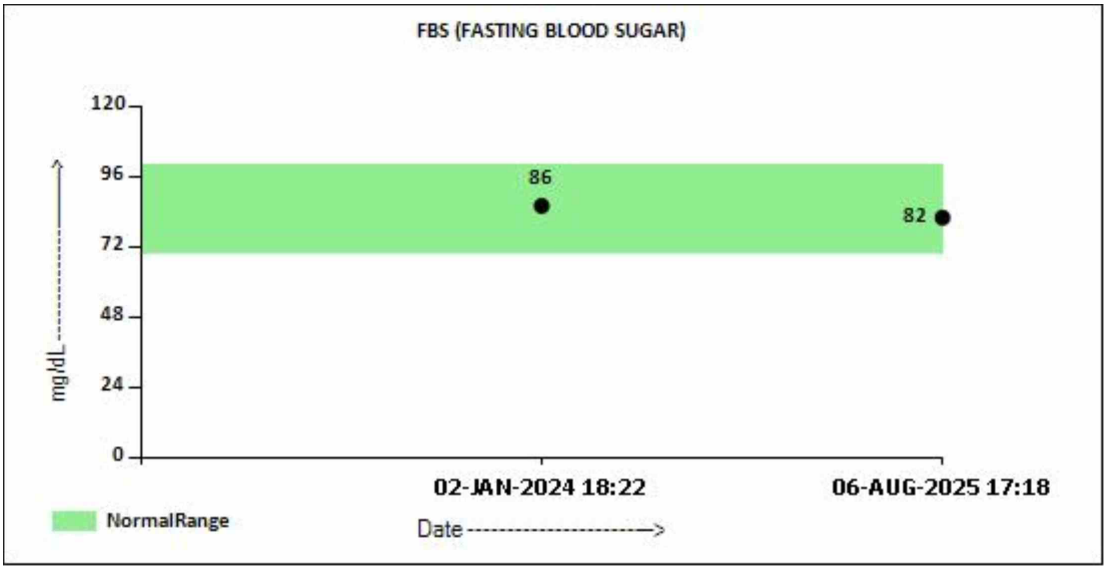
|---|---|---|---|

**BIOCHEMISTRY**

**ANNUAL HEALTH CHECK-UP PACKAGE**

| GLUCOSE FASTING,FLUORIDE PLASMA | Results | 0282XA000026 02/01/24 09:24 | | | Biological Reference Interval | Units |
|---|---|---|---|---|---|---|
| FBS (FASTING BLOOD SUGAR) | 82 | 86 | | | (Normal <100,Impaired fasting glucose:100 to 125,Diabetes mellitus:>=126(on more than 1 occasion)(ADA guidelines 2024) | mg/dL |

METHOD : SPECTROPHOTOMETRY HEXOKINASE




| LIVER FUNCTION PROFILE, SERUM | Results | 0282XA000026 02/01/24 09:24 | | | Biological Reference Interval | Units |
|---|---|---|---|---|---|---|
| BILIRUBIN, TOTAL | 0.5 | | | | Upto 1.2 | mg/dL |

**Dr.Rashmi Rasi Datta-MD,FIMSA DMC-64289**
**Consultant Biochemist & Section Head**

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|---|---|---|---|
| METHOD : COLORIMETRIC DIAZO METHOD | | | |
| BILIRUBIN, DIRECT | 0.2 | < or = 0.3 | mg/dL |
| METHOD : COLORIMETRIC DIAZO METHOD | | | |
| BILIRUBIN, INDIRECT | 0.30 | 0.1 - 1.0 | mg/dL |
| METHOD : CALCULATED PARAMETER | | | |
| TOTAL PROTEIN | 7.1 | 6.0 - 8.0 | g/dL |
| METHOD : SPECTROPHOTOMETRY, BIURET | | | |
| ALBUMIN | 4.6 | 3.97 - 4.94 | g/dL |
| METHOD : SPECTROPHOTOMETRY, BROMOCRESOL GREEN(BCG) - DYE BINDING | | | |
| GLOBULIN | 2.5 | 2.0 - 3.5 | g/dL |
| METHOD : CALCULATED PARAMETER | | | |
| ALBUMIN/GLOBULIN RATIO | 1.8 | 1.0 - 2.1 | RATIO |
| METHOD : CALCULATED PARAMETER | | | |
| ASPARTATE AMINOTRANSFERASE(AST/SGOT) | 37 | < OR = 50 | U/L |
| METHOD : SPECTROPHOTOMETRY, WITH PYRIDOXAL PHOSPHATE ACTIVATION-IFCC | | | |
| ALANINE AMINOTRANSFERASE (ALT/SGPT) | 65 H | < OR = 50 | U/L |
| METHOD : SPECTROPHOTOMETRY, WITH PYRIDOXAL PHOSPHATE ACTIVATION-IFCC | | | |
| ALKALINE PHOSPHATASE | 70 | 40 - 129 | U/L |
| METHOD : SPECTROPHOTOMETRY, PNPP, AMP BUFFER - IFCC | | | |
| GAMMA GLUTAMYL TRANSFERASE (GGT) | 47 | 0 - 60 | U/L |
| METHOD : ENZYMATIC COLORIMETRIC ASSAY STANDARDIZED AGAINST IFCC / SZASZ | | | |
| LACTATE DEHYDROGENASE | 160 | 125 - 220 | U/L |
| METHOD : SPECTROPHOTOMETRY, LACTATE TO PYRUVATE - UV-IFCC | | | |

| **KIDNEY FUNCTION TEST, SERUM** | **Results** | **0282XA000026**<br>02/01/24 09:24 | **Biological Reference Interval** | **Units** |
|---|---|---|---|---|
| BLOOD UREA NITROGEN | 9.1 | 6 - 20 | mg/dL |
| METHOD : SPECTROPHOTOMETRY, KINETIC TEST WITH UREASE AND GLUTAMATE DEHYDROGENASE | | | |



**Dr.Rashmi Rasi Datta-MD,FIMSA**
**DMC-64289**
**Consultant Biochemist & Section Head**

**Dr. Prajapati Kamlesh Ishwarbhai**
**Consultant - Pathologist**

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**PATIENT NAME : TARUN KUMAR** **REF. DOCTOR :** SELF

| | | |
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| **Test Report Status** **Final** | **Results** | **Biological Reference Interval** | **Units** |
|---|---|---|---|
| CREATININE | 0.76 L | 0.90 - 1.30 | mg/dL |
| METHOD : SPECTROPHOTOMETRIC, JAFFE'S KINETICS | | | |
| BUN/CREAT RATIO | 12.04 | 8.0 - 15.0 | |
| METHOD : CALCULATED PARAMETER | | | |
| URIC ACID | 7.0 | 3.4 - 7.0 | mg/dL |
| METHOD : SPECTROPHOTOMETRY, URICASE | | | |
| TOTAL PROTEIN | 7.1 | 6.0 - 8.0 | g/dL |
| METHOD : SPECTROPHOTOMETRY, BIURET | | | |
| ALBUMIN | 4.6 | 3.97 - 4.94 | g/dL |
| METHOD : SPECTROPHOTOMETRY, BROMOCRESOL GREEN(BCG) - DYE BINDING | | | |
| GLOBULIN | 2.5 | 2.0 - 3.5 | g/dL |
| METHOD : CALCULATED PARAMETER | | | |
| CALCIUM | 9.6 | 8.6 - 10.0 | mg/dL |
| METHOD : SPECTROPHOTOMETRY, NM - BAPTA | | | |
| SODIUM, SERUM | 140 | 135 - 145 | mmol/L |
| METHOD : ISE INDIRECT | | | |
| POTASSIUM, SERUM | 4.4 | 3.5 - 5.3 | mmol/L |
| METHOD : ISE INDIRECT | | | |
| CHLORIDE, SERUM | 103 | 97 - 110 | mmol/L |
| METHOD : ISE INDIRECT | | | |

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Tel : 9111591115, Fax : CIN - U74899PB1995PLC045956

**ULR No.775000013678494-0009**

| PATIENT NAME : TARUN KUMAR | | REF. DOCTOR : SELF |
|---|---|---|
| **CODE/NAME & ADDRESS** : C000133041<br>CHECKMED PRIVATE LIMITED<br>SECOND FLOOR, SCO 334-334, SECTOR 35,CHANDIGARH<br>CHANDIGARH 160022<br>9914256267 | ACCESSION NO : **0282YH000247**<br>PATIENT ID : TARUM230691282<br>CLIENT PATIENT ID:<br>ABHA NO : | AGE/SEX : 34 Years Male<br>DRAWN :<br>RECEIVED : 06/08/2025 13:23:28<br>REPORTED : 07/08/2025 09:54:06 |

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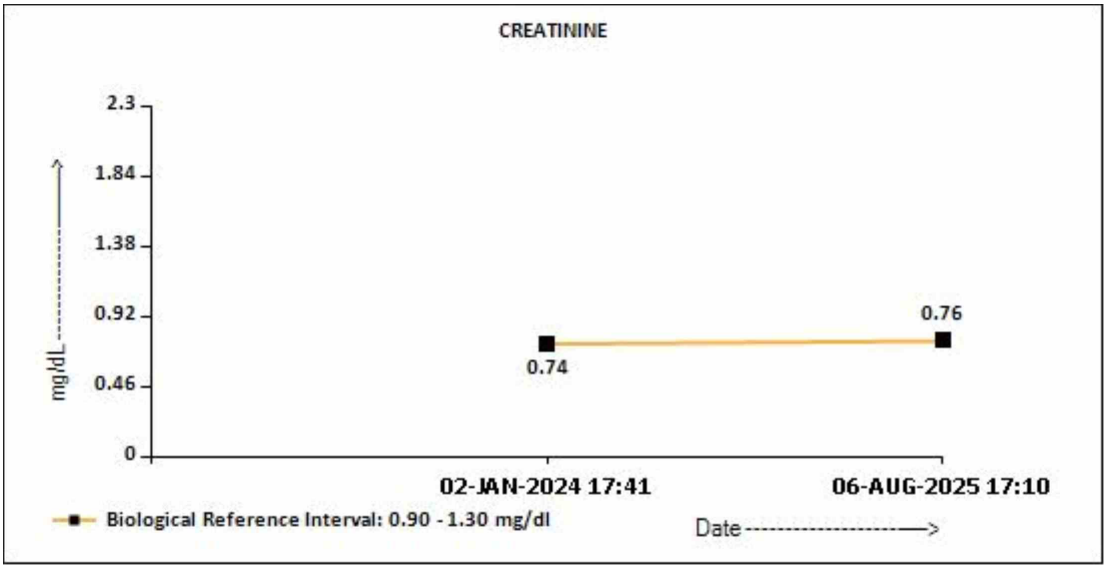


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**Interpretation(s)**

GLUCOSE FASTING,FLUORIDE PLASMA-**TEST DESCRIPTION**

Normally, the glucose concentration in extracellular fluid is closely regulated so that a source of energy is readily available to tissues and sothat no glucose is excreted in the urine.

**Increased in**:Diabetes mellitus, Cushing' s syndrome (10 – 15%), chronic pancreatitis (30%). Drugs:corticosteroids,phenytoin, estrogen, thiazides.

**Decreased in** :Pancreatic islet cell disease with increased insulin,insulinoma,adrenocortical insufficiency,hypopituitarism,diffuse liver disease, malignancy(adrenocortical,stomach,fibrosarcoma),infant of a diabetic mother,enzyme deficiency diseases(e.g.galactosemia),Drugs-insulin,ethanol,propranolol sulfonylureas,tolbutamide,and other oral hypoglycemic agents.

**NOTE:** While random serum glucose levels correlate with home glucose monitoring results (weekly mean capillary glucose values),there is wide fluctuation within individuals.Thus, glycosylated hemoglobin(HbA1c) levels are favored to monitor glycemic control.

High fasting glucose level in comparison to post prandial glucose level may be seen due to effect of Oral Hypoglycaemics & Insulin treatment,Renal Glyosuria,Glycaemic index & response to food consumed,Alimentary Hypoglycemia,Increased insulin response & sensitivity etc.

LIVER FUNCTION PROFILE, SERUM-

**Bilirubin** is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Bilirubin is excreted in bile and urine, and elevated levels may give yellow discoloration in jaundice.**Elevated levels** results from increased bilirubin production (eg, hemolysis and ineffective erythropoiesis), decreased bilirubin excretion (eg, obstruction and hepatitis), and abnormal bilirubin metabolism (eg, hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in Viral hepatitis, Drug reactions, Alcoholic liver disease Conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones getting into the bile ducts, tumors &Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of Hemolytic or pernicious anemia, Transfusion reaction & a common metabolic condition termed Gilbert syndrome, due to low levels of the enzyme that attaches sugar molecules to bilirubin.

**AST** is an enzyme found in various parts of the body. AST is found in the liver, heart, skeletal muscle, kidneys, brain, and red blood cells, and it is commonly measured clinically as a marker for liver health. AST levels increase during chronic viral hepatitis, blockage of the bile duct, cirrhosis of the liver,liver cancer,kidney failure,hemolytic anemia,pancreatitis,hemochromatosis. AST levels may also increase after a heart attack or strenuous activity.ALT test measures the amount of this enzyme in the blood.ALT is found mainly in the liver, but also in smaller amounts in the kidneys,heart,muscles, and pancreas.It is commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health.AST levels increase during acute hepatitis,sometimes due to a viral infection,ischemia to the liver,chronic hepatitis,obstruction of bile ducts,cirrhosis.

**ALP** is a protein found in almost all body tissues.Tissues with higher amounts of ALP include the liver,bile ducts and bone.Elevated ALP levels are seen in Biliary obstruction,



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Osteoblastic bone tumors, osteomalacia, hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, Pagets disease,Rickets,Sarcoidosis etc. Lower-than-normal ALP levels seen in Hypophosphatasia,Malnutrition,Protein deficiency,Wilsons disease.

**GGT** is an enzyme found in cell membranes of many tissues mainly in the liver,kidney and pancreas.It is also found in other tissues including intestine,spleen,heart, brain and seminal vesicles.The highest concentration is in the kidney,but the liver is considered the source of normal enzyme activity.Serum GGT has been widely used as an index of liver dysfunction.Elevated serum GGT activity can be found in diseases of the liver,biliary system and pancreas.Conditions that increase serum GGT are obstructive liver disease,high alcohol consumption and use of enzyme-inducing drugs etc.

**Total Protein** also known as total protein,is a biochemical test for measuring the total amount of protein in serum.Protein in the plasma is made up of albumin and globulin.Higher-than-normal levels may be due to:Chronic inflammation or infection,including HIV and hepatitis B or C,Multiple myeloma,Waldenstroms disease.Lower-than-normal levels may be due to: Agammaglobulinemia,Bleeding (hemorrhage),Burns,Glomerulonephritis,Liver disease, Malabsorption,Malnutrition,Nephrotic syndrome,Protein-losing enteropathy etc.

**Albumin** is the most abundant protein in human blood plasma.It is produced in the liver.Albumin constitutes about half of the blood serum protein.Low blood albumin levels (hypoalbuminemia) can be caused by:Liver disease like cirrhosis of the liver, nephrotic syndrome,protein-losing enteropathy,Burns,hemodilution,increased vascular permeability or decreased lymphatic clearance,malnutrition and wasting etc



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## BIOCHEMISTRY - LIPID

**ANNUAL HEALTH CHECK-UP PACKAGE**

| LIPID PROFILE WITH CALCULATED LDL,SERUM | Results | 0282XA000026 02/01/24 09:24 | | | Biological Reference Interval | Units |
|---|---|---|---|---|---|---|
| CHOLESTEROL, TOTAL<br>METHOD : ENZYMATIC COLORIMETRIC ASSAY | 185 | | | | Desirable : < 200<br>Borderline : 200 - 239<br>High : > / = 240 | mg/dL |
| TRIGLYCERIDES<br>METHOD : ENZYMATIC COLORIMETRIC ASSAY | 142 | | | | Normal: < 150<br>Borderline high: 150 - 199<br>High: 200 - 499<br>Very High: >/= 500 | mg/dL |
| HDL CHOLESTEROL<br>METHOD : HOMOGENEOUS ENZYMATIC COLORIMETRIC ASSAY | 33 L | | | | At Risk: < 40<br>Desirable: > or = 60 | mg/dL |
| CHOLESTEROL LDL<br>METHOD : CALCULATED PARAMETER | 124 H | | | | Adult levels:<br>Optimal < 100<br>Near optimal/above optimal: 100-129<br>Borderline high : 130-159<br>High : 160-189<br>Very high : = 190 | mg/dL |
| NON HDL CHOLESTEROL<br>METHOD : CALCULATED PARAMETER | 152 H | | | | Desirable : < 130<br>Above Desirable : 130 -159<br>Borderline High : 160 - 189<br>High : 190 - 219<br>Very high : > / = 220 | mg/dL |
| VERY LOW DENSITY LIPOPROTEIN<br>METHOD : CALCULATED PARAMETER | 28.4 | | | | < OR = 30.0 | mg/dL |



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|---|---|---|---|---|---|---|
| CHOL/HDL RATIO | 5.6 H | | | | Low Risk : 3.3 - 4.4<br>Average Risk : 4.5 - 7.0<br>Moderate Risk : 7.1 - 11.0<br>High Risk : > 11.0 | |
| METHOD : CALCULATED PARAMETER | | | | | | |
| LDL/HDL RATIO | 3.7 H | | | | 0.5 - 3.0 Desirable/Low Risk<br>3.1 - 6.0 Borderline/Moderate Risk<br>>6.0 High Risk | |
| METHOD : CALCULATED PARAMETER | | | | | | |

**Interpretation(s)**

Serum lipid profile is measured for cardiovascular risk prediction. Lipid Association of India recommends LDL-C as primary target and Non HDL-C as co-primary treatment target.

**Risk Stratification for ASCVD (Atherosclerotic cardiovascular disease) by Lipid Association of India**

| **Risk Category** | |
|---|---|
| Extreme risk group | A.CAD with > 1 feature of high risk group |
| | B. CAD with > 1 feature of Very high risk group or recurrent ACS (within 1 year) despite LDL-C < or = 50 mg/dl or polyvascular disease |
| Very High Risk | 1. Established ASCVD 2. Diabetes with 2 major risk factors or evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia |
| High Risk | 1. Three major ASCVD risk factors. 2. Diabetes with 1 major risk factor or no evidence of end organ damage. 3. CKD stage 3B or 4. 4. LDL >190 mg/dl 5. Extreme of a single risk factor. 6. Coronary Artery Calcium - CAC >300 AU. 7. Lipoprotein a >/= 50mg/dl 8. Non stenotic carotid plaque |
| Moderate Risk | 2 major ASCVD risk factors |
| Low Risk | 0-1 major ASCVD risk factors |

| **Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors** | |
|---|---|
| 1. Age > or = 45 years in males and > or = 55 years in females | 3. Current Cigarette smoking or tobacco use |
| 2. Family history of premature ASCVD | 4. High blood pressure |
| 5. Low HDL | |

**Newer treatment goals and statin initiation thresholds based on the risk categories proposed by LAI in 2020.**

| **Risk Group** | **Treatment Goals** | | **Consider Drug Therapy** | |
|---|---|---|---|---|
| | **LDL-C (mg/dl)** | **Non-HDL (mg/dl)** | **LDL-C (mg/dl)** | **Non-HDL (mg/dl)** |
| Extreme Risk Group Category A | <50 (Optional goal < OR = 30 ) | < 80 (Optional goal <OR = 60) | >OR = 50 | >OR = 80 |
| Extreme Risk Group Category B | <OR = 30 | <OR = 60 | > 30 | >60 |

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|---|---|---|---|

| | | | | |
|---|---|---|---|---|
| Very High Risk | <50 | <80 | >OR= 50 | >OR= 80 |
| High Risk | <70 | <100 | >OR= 70 | >OR= 100 |
| Moderate Risk | <100 | <130 | >OR= 100 | >OR= 130 |
| Low Risk | <100 | <130 | >OR= 130* | >OR= 160 |

*After an adequate non-pharmacological intervention for at least 3 months.

**References:** Management of Dyslipidaemia for the Prevention of Stroke: Clinical Practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.

Interpretation of Diagnostic Tests by Jacques Wallach, 11th Ed.

Friedewald, W. T., Levy, R. I., & Fredrickson, D. S. (1972). "Estimation of the concentration of low-density lipoprotein cholesterol in plasma, without use of the preparative ultracentrifuge."Clinical Chemistry, 18(6), 499-502.

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Third Report of the National Cholesterol Education Program (NCEP) Expert Panel on Detection, Evaluation, and Treatment of High Blood Cholesterol in Adults (Adult Treatment Panel III) Final Report. Circulation, 106(25), 3143-3421.

American College of Cardiology (ACC) and American Heart Association (AHA) (2013). 2013 ACC/AHA Guideline on the Assessment of Cardiovascular Risk. Circulation, 129(25), S49-S73.

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**NEPHELOMETRY**

**ANNUAL HEALTH CHECK-UP PACKAGE**

| C-REACTIVE PROTEIN, SERUM (QUANTITATIVE) | Results | 0282XA000026<br>02/01/24 09:24 | | | Biological Reference Interval | Units |
|---|---|---|---|---|---|---|
| C-REACTIVE PROTEIN | 1.3 | | | | < 5.0 | mg/L |

METHOD : PARTICLE ENHANCED IMMUNOTURBIDIMETRIC ASSAY

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**Interpretation(s)**
C-REACTIVE PROTEIN, SERUM (QUANTITATIVE)- **Test Description:**
A CRP test measures the amount of CRP in the blood to detect inflammation due to acute conditions or to monitor the severity of disease in chronic conditions. CRP is one of the proteins commonly referred to as acute phase reactants. CRP is distinguished by its rapid response to trauma or infection.Synthesis of CRP increases within 4-6 hours of onset of inflammation, reaching peak values within 1-2 days. CRP levels also fall quickly after resolution of inflammation since its half life is 6 hours.
This standard CRP test is not to be confused with a hs-CRP test. These are two different tests that measure CRP and each test measures a different range of CRP levels in the blood for different purposes. The standard CRP test measures high levels of protein observed in diseases that cause significant inflammation.
**Test Interpretation:**
Increased CRP level: Increasing amount of CRP in the blood suggests the presence of inflammation but will not identify its location or the cause.
Suspected bacterial infection: a high CRP level can confirm that you have a serious bacterial infection.
Chronic inflammatory disease: high levels of CRP suggest a flare-up if you have a chronic inflammatory disease or that treatment has not been effective.
Testing for CRP is indicated in the following clinical situations - monitoring recovery from surgery, myocardial infarction, transplantation, inflammatory bowel disease, rheumatic diseases and infectious diseases. Measuring and charting C-reactive protein values can also prove useful in determining disease progress or the effectiveness of treatments
CRP levels can be elevated in the later stages of pregnancy as well as with the use of birth control pills or hormone replacement therapy (i.e., estrogen). Higher levels of CRP have also been observed in people who are obese. CRP can also be increased in people who have cancer.
**Recommendation:** The hs-CRP test precisely detects lower levels of the protein than that measured by the standard CRP test and is also used to evaluate individuals for risk of cardiovascular disease. It measures CRP in the range from 0.15 to 20 mg/L.
**Limitation:**
CRP levels in autoimmune diseases may show little or no increase unless infection is present. Levels may not increase in conditions like pregnancy, angina, seizures, asthma, common cold. The main limitation of CRP is in its non-specific response and should not be interpreted without a complete clinical history and evaluation.



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**SEROLOGY**

**ANNUAL HEALTH CHECK-UP PACKAGE**

**VDRL,SERUM**

| | | | |
|---|---|---|---|
| VDRL (MODIFIED) | NON REACTIVE | NONREACTIVE | TITER |

METHOD : FLOCCULATION PRINCIPLE (TRUST)

**Interpretation(s)**
VDRL,SERUM-**Test description:-**
VDRL/RPR/TRUST are Non-treponemal screening tests for syphilis.They achieve a sensitivity of 100% in the detection of secondary syphilis.After the secondary stage, titres dimish and about a third of patients with late syphilis are seronegative.The titres may rise in patients developing cardiovascular, neurological or gummatous lesions. The tests usually become negative 6 to 18 months after effective treatment of syphilis, depending on the stage at which treatment is intitiated.If treatment is started late, tests may remain positive in low titres.

**Interpretation:-**
Diagnostic titre for VDRL is >=1:8. All Titres < 1:8 must be confirmed with a treponemal test (TPHA) for diagnosing syphilis,as it may be a biological false positive result.

**Limitations:-**
Nontreponemal tests lack sensitivity in primary and late syphilis.False negative reactions can occur in stages of the disease where there is minimal tissue damage, particularly in early infection and in latent stages.
Biologic false positive reactions are common in a variety of other infections (hepatitis, Leprosy,Malaria,Relapsing fever,Infectious mononucleosis,hepatits), Rheumatic diseases and Auto- Immune disorders and may persist upto 6 months. Chronic false-positive reactions (persisting beyond 6 months) may be seen in connective tissue diseases ,immunoglobulin abnormalities, narcotic addiction, leprosy, and malignancy.
More specific Treponemal tests, such as Treponema pallidum Hemagglutination assay (TPHA) test are recommended for confirmation.

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## CLINICAL PATH - URINALYSIS

**ANNUAL HEALTH CHECK-UP PACKAGE**

**PHYSICAL EXAMINATION, URINE**

COLOR — PALE YELLOW
METHOD : MANUAL

APPEARANCE — CLEAR
METHOD : MANUAL

| CHEMICAL EXAMINATION, URINE | Results | 0282XA000026<br>02/01/24 09:24 | | | Biological Reference Interval | Units |
|---|---|---|---|---|---|---|
| PH | <=5.0 | 5.5 | | | 4.5 - 7.5 | |
| METHOD : DOUBLE INDICATOR PRINCIPLE | | | | | | |
| SPECIFIC GRAVITY | 1.025 | >=1.030 | | | 1.005 - 1.030 | |
| METHOD : IONIC CONCENTRATION METHOD/DIPSTICK | | | | | | |
| PROTEIN | NOT DETECTED | | | | NOT DETECTED | |
| METHOD : PROTEIN-ERROR-OF INDICATORS PRINCIPLE | | | | | | |
| GLUCOSE | NOT DETECTED | | | | NEGATIVE | |
| METHOD : PEROXIDASE / MANUAL | | | | | | |
| KETONES | NOT DETECTED | | | | NOT DETECTED | |
| METHOD : NITROPRUSSIDE REACTION / MANUAL | | | | | | |
| BLOOD | NOT DETECTED | | | | NEGATIVE | |
| METHOD : DISOPROPLBENZENE / MANUAL | | | | | | |
| BILIRUBIN | NOT DETECTED | | | | NOT DETECTED | |
| UROBILINOGEN | Nor | | | | NORMAL | |
| METHOD : DIETHYLAMINOBENZ ALDEHYDE / MANUAL | | | | | | |

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| NITRITE | NOT DETECTED | NOT DETECTED | |
| METHOD : DIAZONIUM COMPOUND METHOD | | | |
| LEUKOCYTE ESTERASE | NOT DETECTED | NOT DETECTED | |
| METHOD : ESTERASES/DIPSTICK | | | |

**MICROSCOPIC EXAMINATION, URINE**

| | | | |
|---|---|---|---|
| RED BLOOD CELLS | NOT DETECTED | 0 - 2 | /HPF |
| METHOD : MICROSCOPY | | | |
| PUS CELL (WBCS) | 0-1 | 0-5 | /HPF |
| METHOD : MICROSCOPY | | | |
| EPITHELIAL CELLS | 0-1 | 0-5 | /HPF |
| METHOD : MICROSCOPY | | | |
| CASTS | NOT DETECTED | | |
| METHOD : MICROSCOPY | | | |
| CRYSTALS | NOT DETECTED | | |
| METHOD : MICROSCOPY | | | |
| BACTERIA | NOT DETECTED | NOT DETECTED | |
| METHOD : MICROSCOPY | | | |
| YEAST | NOT DETECTED | NOT DETECTED | |
| METHOD : MICROSCOPY | | | |

**Interpretation(s)**

The following table describes the probable conditions, in which the analytes are present in urine

| Presence of | Conditions |
|---|---|
| Proteins | Inflammation or immune illnesses |
| Pus (White Blood Cells) | Urinary tract infection, urinary tract or kidney stone, tumors or any kind of kidney impairment |
| Glucose | Diabetes or kidney disease |
| Ketones | Diabetic ketoacidosis (DKA), starvation or thirst |
| Urobilinogen | Liver disease such as hepatitis or cirrhosis |

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| | |
|---|---|
| Blood | Renal or genital disorders/trauma |
| Bilirubin | Liver disease |
| Erythrocytes | Urological diseases (e.g. kidney and bladder cancer, urolithiasis), urinary tract infection and glomerular diseases |
| Leukocytes | Urinary tract infection, glomerulonephritis, interstitial nephritis either acute or chronic, polycystic kidney disease, urolithiasis, contamination by genital secretions |
| Epithelial cells | Urolithiasis, bladder carcinoma or hydronephrosis, ureteric stents or bladder catheters for prolonged periods of time |
| | |
| Granular Casts | Low intratubular pH, high urine osmolality and sodium concentration, interaction with Bence-Jones protein |
| Hyaline casts | Physical stress, fever, dehydration, acute congestive heart failure, renal diseases |
| Calcium oxalate | Metabolic stone disease, primary or secondary hyperoxaluria, intravenous infusion of large doses of vitamin C, the use of vasodilator naftidrofuryl oxalate or the gastrointestinal lipase inhibitor orlistat, ingestion of ethylene glycol or of star fruit (Averrhoa carambola) or its juice |
| Uric acid | arthritis |
| Bacteria | Urinary infectionwhen present in significant numbers & with pus cells. |
| Trichomonas vaginalis | Vaginitis, cervicitis or salpingitis |

Reference:

Henrys Clinical Diagnostics and Management by Laboratory Methods 21st edition (pg 410).

***This is a tabular depiction of test results illustrating values of the previous three reports of the patient which needs to be correlated with the original reports**

K. I. Prajapati



**Dr. Prajapati Kamlesh Ishwarbhai**
**Consultant - Pathologist**

View Details View Report

| | | |
|---|---|---|
| **PATIENT NAME : TARUN KUMAR** | **REF. DOCTOR :** SELF | |
| **CODE/NAME & ADDRESS :** C000133041<br>CHECKMED PRIVATE LIMITED<br>SECOND FLOOR, SCO 334-334, SECTOR 35,CHANDIGARH<br>CHANDIGARH 160022<br>9914256267 | ACCESSION NO : **0282YH000247**<br>PATIENT ID : TARUM230691282<br>CLIENT PATIENT ID:<br>ABHA NO : | AGE/SEX : 34 Years Male<br>DRAWN :<br>RECEIVED : 06/08/2025 13:23:28<br>REPORTED : 07/08/2025 09:54:06 |

**Test Report Status** **Final** **Results** **Biological Reference Interval** **Units**

**SPECIALISED CHEMISTRY - HORMONE**

**ANNUAL HEALTH CHECK-UP PACKAGE**

| THYROID PANEL, SERUM | Results | 0282XA000026<br>02/01/24 09:24 | | | Biological Reference Interval | Units |
|---|---|---|---|---|---|---|
| T3 | 156.0 | | | | 80 - 200 | ng/dL |
| METHOD : ELECTROCHEMILUMINESCENCE IMMUNO ASSAY | | | | | | |
| T4 | 9.63 | | | | 5.1 - 14.1 | µg/dL |
| METHOD : ELECTROCHEMILUMINESCENCE IMMUNO ASSAY | | | | | | |
| TSH (ULTRASENSITIVE) | 1.980 | | | | 0.270 - 4.200 | µIU/mL |
| METHOD : ELECTROCHEMILUMINESCENCE IMMUNO ASSAY | | | | | | |

**Interpretation(s)**

**Triiodothyronine T3** , **Thyroxine T4,** and **Thyroid Stimulating Hormone TSH** are thyroid hormones which affect almost every physiological process in the body, including growth, development, metabolism, body temperature, and heart rate.
Production of T3 and its prohormone thyroxine (T4) is activated by thyroid-stimulating hormone (TSH), which is released from the pituitary gland. Elevated concentrations of T3, and T4 in the blood inhibit the production of TSH.
Excessive secretion of thyroxine in the body is hyperthyroidism, and deficient secretion is called hypothyroidism.
In primary hypothyroidism, TSH levels are significantly elevated, while in secondary and tertiary hyperthyroidism, TSH levels are low.
Below mentioned are the guidelines for Pregnancy related reference ranges for Total T4, TSH & Total T3.Measurement of the serum TT3 level is a more sensitive test for the diagnosis of hyperthyroidism, and measurement of TT4 is more useful in the diagnosis of hypothyroidism.Most of the thyroid hormone in blood is bound to transport proteins. Only a very small fraction of the circulating hormone is free and biologically active. It is advisable to detect Free T3, FreeT4 along with TSH, instead of testing for albumin bound Total T3, Total T4**.**

| Sr. No. | TSH | Total T4 | FT4 | Total T3 | Possible Conditions |
|---|---|---|---|---|---|
| 1 | High | Low | Low | Low | (1) Primary Hypothyroidism (2) Chronic autoimmune Thyroiditis (3) Post Thyroidectomy (4) Post Radio-Iodine treatment |
| 2 | High | Normal | Normal | Normal | (1)Subclinical Hypothyroidism (2) Patient with insufficient thyroid hormone replacement therapy (3) In cases of Autoimmune/Hashimoto thyroiditis (4). Isolated increase in TSH levels can be due to Subclinical inflammation, drugs like amphetamines, Iodine containing drug and dopamine antagonist e.g. domperidone and other physiological reasons. |
| 3 | Normal/Low | Low | Low | Low | (1) Secondary and Tertiary Hypothyroidism |

**Dr.Rashmi Rasi Datta-MD,FIMSA**
**DMC-64289**
**Consultant Biochemist & Section Head**

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**ULR No.775000013678494-0009**

**Test Report Status** **Final** **Results** **Biological Reference Interval** **Units**

| | | | | | |
|---|---|---|---|---|---|
| 4 | Low | High | High | High | (1) Primary Hyperthyroidism (Graves Disease) (2) Multinodular Goitre (3)Toxic Nodular Goitre (4) Thyroiditis (5) Over treatment of thyroid hormone (6) Drug effect e.g. Glucocorticoids, dopamine, T4 replacement therapy (7) First trimester of Pregnancy |
| 5 | Low | Normal | Normal | Normal | (1) Subclinical Hyperthyroidism |
| 6 | High | High | High | High | (1) TSH secreting pituitary adenoma (2) TRH secreting tumor |
| 7 | Low | Low | Low | Low | (1) Central Hypothyroidism (2) Euthyroid sick syndrome (3) Recent treatment for Hyperthyroidism |
| 8 | Normal/Low | Normal | Normal | High | (1) T3 thyrotoxicosis (2) Non-Thyroidal illness |
| 9 | Low | High | High | Normal | (1) T4 Ingestion (2) Thyroiditis (3) Interfering Anti TPO antibodies |

REF: 1. TIETZ Fundamentals of Clinical chemistry 2.Guidlines of the American Thyroid association duriing pregnancy and Postpartum, 2011.

**TSH in pregnancy**

There's reduction in both the lower and the upper limit of maternal TSH relative to the non-pregnant TSH reference range. This is because of elevated levels of serum hCG that directly stimulates the TSH receptor, thereby increasing thyroid hormone production. The largest decrease in serum TSH is observed during the first trimester. Thereafter, serum TSH and its reference range gradually increases in the second and third trimesters, but nonetheless remains lower than in non-pregnant women.

**NOTE: It is advisable to detect Free T3,FreeT4 along with TSH, instead of testing for albumin bound Total T3, Total T4.**TSH is not affected by variation in thyroid - binding protein. TSH has a diurnal rhythm, with peaks at 2:00 - 4:00 a.m. And troughs at 5:00 - 6:00 p.m. With ultradian variations.

***This is a tabular depiction of test results illustrating values of the previous three reports of the patient which needs to be correlated with the original reports**



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